

Patient: **LAMBERT III, ANTONIO M**

MRN: 051483660

Admit: 8/7/2026

Disch: 8/7/2026

FIN: 1206634254

PCP Smith CRNP,Susan R

DOB/Age 6/22/2020 6 years

Attending: Rowell Jr MD,Frederick W

Gender Male

Ordering n/a

Location: CW Peds 4 North; 4760; 01

Receiving: HealtheLife,Portal

Patient Viewable Clinical Documents

Document Type:

Discharge Summary

Service Date/Time:

8/7/2026 16:46 CDT

Result Status:

Auth (Verified)

Document Subject:

Pediatric Discharge Summary

Sign Information:

Rowell Jr MD,Frederick W (8/7/2026 19:38 CDT); Acevedo Echevarria MD,Jose M (8/7/2026 16:46 CDT)

Patient Name: LAMBERT III, ANTONIO M

Date of Birth: 06/22/2020

Admission Date: 08/07/2026

Discharge Diagnoses

1. Convulsive syncope, Brief loss of consciousness

Iron deficiency anemia, Discharge, 08/07/2026

Admission Information

Antonio is a 6-year-old male with a past medical history of Iron Deficiency Anemia, referred from Baldwin health ED for seizure-like activity.

Patient was in his usual state of health until around 6:30 PM on 8/6/26 when he was with his paternal grandmother and aunt at the OWA amusement park. Grandmother noted that he ate a hot dog shortly before going down a water slide. Then, when she got up to check on him, she saw that he was at the bottom of the slide and not moving away. She tried calling the patient's name but there was no response. When she approached at a closer distance, she noted that his entire body was shaking with associated eye rolling.

This episode lasted about 1 minute. Patient did not appear to be breathing at the time and CPR was *allegedly* initiated by 2 bystanders. There was no fecal incontinence but urinary incontinence cannot be ruled out as patient was already submerged in water. No lingula trauma. Denies recent history of fever, cough, nasal congestion, vomiting, diarrhea, headache, chest pain.

Pre-hospital course:

EMS activated to transport patient from OWA water park. Patient was slightly altered immediately after seizure, with low O2 readings in the 80s and supplemental oxygen given. However, his mental status and hypoxia improved and he returned to baseline prior to hospital arrival.

Baldwin Health ED course:

At the ED his GCS was 15, BP- 98/72; P-111; R-26, Spo2-100% on RA

USA Childrens and Womens Hospital

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Physical examination normal

Labs: Hb-10.8, HCT-32.9, WBC-7.2, Plt- 137

Na- 142, K-3.9, Cl-111, CO2-21, Glu-97, BUN-10, Cr-0.5

Imaging: Chest X-ray normal; CT brain w/o contrast- normal

Interventions: No rescue seizure medication required

Birth Hx: Born via SVD at 38 weeks gestation; maternal and a perinatal history benign

Immunizations: Up-to-date

PMH: No previous hospitalizations, Iron Deficiency Anemia

PSH: Nil

Allergies: NKDA

Medications: Nil

Family Hx: Father has history of 1 seizure during childhood

Hypertension (maternal grandmother)

Asthma (maternal grandmother)

Diabetes melitis, lung cancer, breast cancer

Social Hx: Lives with mom, no pets, moderate amenities present, no food insecurity

PCP: Susan Smith, CRNP

Pharmacy: Walgreens, 370 Schillinger Road

Hospital Course

Patient was admitted for concerns of seizures. Neurology evaluated patient and stated that patient's symptoms most likely convulsive syncope. Orthostatic vitals were obtained and were unremarkable. CMP was obtained due to concerns of dehydration and was unremarkable. CBC showed microcytic anemia and iron studies showed low iron. Given symptoms not likely epileptic in nature and patient neurologic exam reassuring will not start AEDs. At time of discharge vital signs were stable and physical exam was unremarkable. Patient was discharged home with iron supplementation.

Quality Measures

No qualifying data available.

Vital Signs

T: 36.5 °C **T:** 36.5 °C (Oral) **TMIN:** 36.5 °C (Oral)

TMAX: 36.5 °C **HR:** 78 (Peripheral) **HR:** 85 (Monitored)

RR: 24 **BP:** 100/53 **SpO2:** 100% **HT:** 118 cm

WT: 19.1 kg (Measured) **BMI:** 13.7

Orthostatic BP **Orthostatic HR**

Supine 90/48 08/07/2026 12:40 CDT 88 08/07/2026 12:40 CDT

Sitting 84/61 08/07/2026 12:40 CDT 98 08/07/2026 12:40 CDT

Standing 95/72 08/07/2026 12:40 CDT 103 08/07/2026 12:40

CDT

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Physical Exam

GEN: Alert, active. No apparent distress. Well developed. Well nourished.

HEENT: Head: Normocephalic/attraumatic.

NECK: Supple. No lymphadenopathy

LUNGS: Clear to auscultation with equal breath sounds. No wheezes, rales or rhonchi.

HEART: Regular rate and rhythm; normal S1/S2. No murmur.

ABDOMEN: Soft, nontender, normal bowel sounds. No hepatosplenomegaly. No masses. No hernia.

Neuro: Alert, oriented, good muscle tone, CN II-XII grossly intact

Discharge Medications

Changed

ferrous sulfate (ferrous sulfate 300 mg/5 mL (60 mg elemental iron) oral liquid) 2 Milliliters Oral 2 times a day. Refills: 0.

Pending Labs/Radiology

.eGFR Pediatric

Automated Diff

CBC w/Diff

CMP

Ferritin

Hematocrit

Hemoglobin

Iron (Fe) Level

TIBC

Plan

1. Convulsive syncope R55, Brief loss of consciousness R55

Iron deficiency anemia D50.9

Ordered:

Clinic Follow Up -- Hospital to Clinic, = 08/07/26 16:24:00 CDT,

1-2 weeks, Smith CRNP, Susan R, Hospital discharge, Iron deficiency anemia

Orders:

ferrous sulfate 300 mg/5 mL (60 mg elemental iron) oral liquid

Discharge Patient, Ordered

6-year-old male with past medical history of iron deficiency anemia admitted for concerns of seizure-like activity.

Iron deficiency anemia

Seizure activity

Syncope

Patient was evaluated by neurology who stated that patient symptoms more likely are due to syncope versus seizure. CMP was done to rule out dehydration and was unremarkable.

Echocardiogram was obtained as part of syncope workup and was unremarkable. EEG was deferred due to low suspicion of seizures.

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CBC showed microcytic anemia and iron studies showed a TSAT of 16%, consistent with iron deficiency anemia. Which could explain patient symptoms.

Plan:

- Ferrous sulfate 3 mg/kg twice daily
- Follow-up with PCP in 1 to 2 weeks

Disposition: Discharge home

Attending: Dr. Rowell

Follow-Up

With	When	Contact Information
Smith CRNP, Susan R, Pediatrics	Within 1 to 2 weeks	1835 Old Shell Rd Mobile, AL 36607- (251)706-8170
Additional Instructions:		

Education Given to Patient

Iron Deficiency Anemia, Pediatric

Patient Instructions

- Follow-up with PCP in 1 to 2 weeks
- Ferrous sulfate 2 mL twice daily. Will need repeat iron studies in 1 month by PCP
- If develop abnormal movements concerning for seizures please bring patient to the emergency department for evaluation.
- If symptoms recur, worsen, develops shortness of breath, dizziness, lightheadedness, lethargy please return to the emergency department for evaluation.

Attending Attestation

Attending Addendum: I have personally interviewed and examined the patient. I discussed the findings, interventions, and diagnostic testing with the house staff. I agree with the findings and treatment plan as documented in the resident's notes with exceptions in our respective documentation. I personally reviewed the pt's laboratory studies and imaging in this note. I spent more than 30 minutes in discharge management/planning.

Electronically Signed on 08/07/26 07:38 PM

Rowell Jr MD, Frederick W

Electronically Signed on 08/07/26 04:46 PM

Acevedo Echevarria MD, Jose M